

Question:

A 5-year-old boy is brought to the physician by his mother because he does not “listen to her” anymore. The mother also reports that her son cannot concentrate on any tasks lasting longer than just a few minutes. Teachers at his preschool report that the patient is more active compared to other preschoolers, frequently interrupts or bothers other children, and is very forgetful. Last year the patient was expelled from another preschool for hitting his teacher and his classmates when he did not get what he wanted and for being disruptive during classes. He was born at term via vaginal delivery and has been healthy except for 3 episodes of acute otitis media at the age of 2 years. He has met all developmental milestones. His mother has major depressive disorder and his father has Graves' disease. He appears healthy and well nourished. Examination shows that the patient does not seem to listen when spoken to directly. The remainder of the examination shows no abnormalities. Which of the following is the most appropriate next step in treatment?

Answer Choices:

- A. Behavior therapy
- B. Methimazole
- C. Fluoxetine
- D. Hearing aids

Reasoning + Answer:

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Finding Reasoning Paths :

- 1 . **In attentiveness + Impulsivity + Disinhibition** : These behaviors suggest a developmental and behavioral disorder , such as attention-deficit/hyperactivity disorder (ADHD), which is common in children .
- 2 . **Motor Restlessness + Inattention + Impulsivity** : These symptoms align with ADHD , which is often treated with behavioral therapy and sometimes stimulant medications like methylphenidate .
- 3 . **Family History of Psychiatric Disorders** : The mother 's major depressive disorder and the father 's Graves ' disease could suggest a genetic predisposition to psychiatric disorders , but they are less likely to directly cause the observed symptoms .
- 4 . **Impairment in Adaptive Behavior** : The child 's hyperactivity , impulsivity , and difficulty concentrating indicate significant functional impairment , which aligns with ADHD .
- 5 . **Educational History** : The expulsion from preschool for disruptive behavior further supports the diagnosis of ADHD , as these are hallmark features of the disorder .

Reasoning Process :

- 1 . The child exhibits **inattentiveness** , **impatience** , **motor restlessness** , **impulsivity** , and **disinhibition** . These behaviors are consistent with **attention-deficit/hyperactivity disorder (ADHD)** , a neurodevelopmental disorder commonly diagnosed in children .
- 2 . The child 's **inattentiveness** and **impatience** limit his ability to concentrate and follow instructions , while **motor restlessness** and **impulsivity** contribute to his hyperactivity and frequent interruption of others . These symptoms are mutually exclusive , ruling out other potential diagnoses like obsessive-compulsive disorder or mood disorders .
- 3 . The child 's **family history** of psychiatric disorders (mother 's major depressive disorder , father 's Graves ' disease) may raise questions about genetic or environmental factors contributing to his symptoms . However , these are likely secondary to the primary diagnosis of ADHD , which is more commonly associated with neurobiological factors .
- 4 . The child 's **educational history** (expulsion from preschool due to disruptive behavior) further supports the likelihood of ADHD , as these behaviors are a hallmark of the disorder .
- 5 . ADHD is often treated with a combination of **behavioral therapy** and **medication** , with stimulant medications like methylphenidate being the most commonly prescribed in younger children . However , methylphenidate is not the first-line treatment ; the gold standard for ADHD is **behavioral therapy** , which addresses the root causes of hyperactivity , impulsivity , and inattention through structured behavioral interventions .

Conclusion :

The most appropriate next step in treatment is **behavioral therapy** , which has been the gold standard for managing ADHD since the 1970s . This approach targets the underlying behavioral deficits and helps the child develop better self-regulation and focus .

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<answer>

Behavior therapy

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